

Response — Request for Medical Information, MSH-INJ-5795 [CLM-317073]

From: Cory Shepherd

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To: LBH Injury Management

lbh_injurymanagement@health.qld.gov.au

Cc: LBH_HR lbh_hr@health.qld.gov.au, Heath

Moran heath.moran@together.org.au

Sent: Monday 3 August at 11:38

Dear Human Resources team,

I refer to the email from Injury Management of Friday 31 July 2026 at 11:43am and the two letters of the same date signed by Mr Scott Hughes, Director, Corporate Services. I have directed this response to Human Resources, and copied Injury Management so that it is received within the timeframe the letter set.

Nothing in this response is a personal complaint about any individual, and I ask for no finding about anyone. My concern is with how the process is being run, and with the support and adjustment that ordinarily apply when an employee is certified fit with restrictions. I have copied my union, and I have written separately to the Chief Executive today about the conflict of interest and the handling of my personal information.

I am engaging with this process and will obtain the information sought. My response is attached, with a proposed allocation of the nine questions and my proposal for return to work. In summary:

- Questions 1(d), 4, 5 and 8 I will obtain from my general practitioner; Questions 7, 1(a), 1(d) and 8 from my treating psychiatrist. Please confirm which questions the Health Service presses, and the invoices will be

directed to you under the arrangement in your letter.

- Questions 2, 3, 6 and 9 are not medical questions: they ask my doctor to answer matters which G03 and the Work Health and Safety Act place with the Health Service. I ask that the Health Service answer those itself; the medical part of each can then be put to my practitioners.
- Question 1(a) is answered openly in the attachment: the diagnosis was made on 24 October 2024, with the clinical context to come from my psychiatrist.
- Question 1(c) I consent to, and the answer is in Dr Ma's own file: his understanding of my condition derives from my psychiatrist's correspondence, not my self-report.
- Question 1(b) alone I ask be withdrawn. G03 clause 3 permits medical information "where necessary" to identify or assess adjustments; a causal formulation does neither, and it goes to a matter in issue in WC/2024/227.

I adopt option 1 and will obtain the information myself. I do not provide an authority under option 2.

Three matters are dealt with in the attachment and I flag them here.

First, timing. I met the 2:00pm deadline of 3 July; the checklist certified me fit with restrictions; within the hour I was told I could not attend, and I have been held out for five weeks without an instrument being identified or a date given. Your letter arrived two hours before a consultation with Dr Ma of which the Health Service had notice; I cancelled it rather than waste a cost, and will rebook once the question set is settled. I ask for the extension your letter contemplates, and for a

date by which the Health Service will tell me whether I am returning to work. I also ask what decision is meant by "a decision regarding your ability to perform your role", and under what instrument it would be made — this is the second occasion an adverse position has been foreshadowed without one.

Second, my exclusion. The Health Service's own letter confirms complaints are escalated to the Client Liaison Officer and/or Manager, and the Role Description contains no complaint-handling duty. There is no identified restriction preventing my performance of the role. I ask to be returned to my roster, and that the leave debited since 3 July 2026 be reversed.

Third, delegation and conflict. I previously asked who is making decisions about me and whether any conflict has been identified and managed; I have not been answered. Part 6 of the attachment asks again and proposes the ordinary separation of functions.

I also attach a short notice setting out the arrangements for the medical appointments — which I will rebook within three business days of the Health Service confirming which questions it presses — and the position on the cost of the reports.

I remain willing to return to work on terms which are safe for me and workable for the Health Service, and would be glad to meet with a support person present. My proposal is attached as a separate document, the Proposal for Return to Work, and I ask that it be received together with my separate request of today under clause 10.3.2 of the certified agreement.

Yours sincerely,

Cory Lea Shepherd

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